



ACCOUNTING POLICY PAPER SCOPE

SOCIAL HEALTH AUTHORITY

22nd May 2026

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IFRS 17 Insurance Contracts establishes principles for the recognition, measurement, presentation, and disclosure of insurance contracts within the scope of the Standard. The objective of IFRS 17 is to ensure that an entity provides relevant information that faithfully represents those contracts. This information gives a basis for users of financial statements to assess the effect that insurance contracts have on the entity's financial position, financial performance, and cash flows.

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DOCUMENT PURPOSE

Submission to the Management of Social Health Authority

DOCUMENT OWNERSHIP

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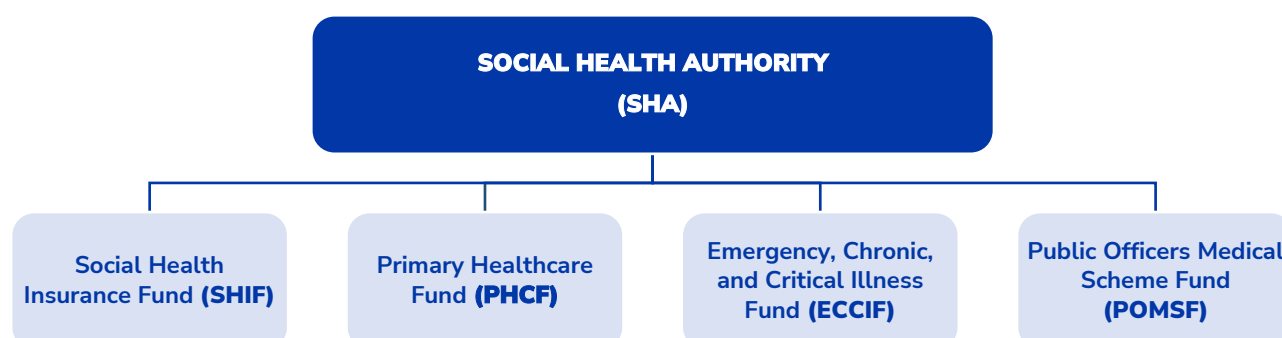
ABBREVIATIONS & DEFINITIONS

NAME	DEFINITION
ECCF	Emergency, Chronic and Critical Illness Fund
IASB	International Accounting Standards Board
IFRS	International Financial Reporting Standard
KAFS	Kenbright Actuarial and Financial Services Limited
NHIF	National Health Insurance Fund
PHCF	Primary Health Care Fund
POMSF	Public Officers Medical Scheme Fund
SHA	Social Health Authority
SHIF	Social Health Insurance Fund
UHC	Universal Health Coverage

1 INTRODUCTION

Background

- 1.1 The International Accounting Standards Board ('IASB') introduced IFRS 17 in May 2017 to supersede IFRS 4 which was an interim standard. IFRS 17 insurance contracts establishes the principles for the recognition, measurement, presentation, and disclosure of insurance contracts.
- 1.2 The objective of IFRS 17 is to ensure that an entity provides relevant information that faithfully represent contracts that fall within the scope of the Standard.
- 1.3 The Social Health Authority ('SHA') is a statutory agency established under Section 4 of the Social Health Insurance Act, which was enacted on 22nd November 2023 to succeed the National Hospital Insurance Fund (NHIF) as Kenya's primary health coverage provider. NHIF ceased its operations on **30th June 2024**, with its final admissions date being 30th September 2024. The transition period, spanning from November 2023 to mid-2024, involved the legal transfer of assets, liabilities and functions. SHA officially commenced its operations on **1st October 2024**.
- 1.4 SHA is mandated to pool contributions from individuals, employers, and the government in order to purchase healthcare services from empaneled and contracted healthcare providers and pay for the provision of quality healthcare services to members and their beneficiaries.
- 1.5 SHA operates under the Social Health Insurance Act and manages the following four funds:



Purpose

- 1.6 This accounting policy paper sets out the position of SHA regarding the application of the Standard to the contracts it administers. The paper provides clarity on the scope of IFRS 17, evaluates whether SHA's contracts fall within it, and outlines the applicable accounting treatment for each contract type.
- 1.7 This guidance is crucial during the implementation of the Standard to ensure accurate classification, compliance, and consistent financial reporting.
- 1.8 By following this policy, SHA can ensure that all relevant contracts are appropriately evaluated and reported in accordance with IFRS 17 requirements, thereby enhancing the transparency and reliability of its financial statements.

Objective

- 1.9 This policy paper provides detailed criteria for determining whether contracts issued and services provided by SHA fall within the scope of IFRS 17.
- 1.10 The paper will also determine whether contracts outside the scope require unbundling to assess whether the components fall within the scope of IFRS 17.
- 1.11 This policy is designed to be seamlessly integrated into SHA's financial reporting framework thereby ensuring that all insurance contracts are evaluated and reported in a manner that aligns with IFRS 17 requirements.

Scope and Transition

- 1.12 This policy applies to all insurance contracts issued by SHA that meet the definition of insurance contracts under IFRS 17.
- 1.13 SHA shall adopt IFRS 17 with its transition date being **1st October 2024**. This transition includes the legal transfer of NHIF's assets, liabilities and functions to SHA, ensuring continuity of healthcare services while expanding coverage and improving access under the new structure.

2 IFRS 17 INSURANCE CONTRACTS

2.1 According to IFRS 17, an insurance contract is defined as

A contract under which one party (the issuer) accepts significant insurance risk from another party (the policyholder) by agreeing to compensate the policyholder if a specified uncertain future event (the insured event) adversely affects the policyholder.

2.2 SHA accepts significant risk from its policyholders by entering into agreements that obligate it to provide financial compensation if an unforeseen insured event has a negative impact on the policyholder.

2.3 SHA must assess whether its contracts meet the definition of insurance contracts under IFRS 17 or whether alternative standards apply.

Technical Analysis

2.4 This subsection focuses on determination of whether an insurance contract falls under IFRS 17 scope by considering the uncertain future event and significant insurance risk.

Uncertain future event

2.5 Transfer of insurance risk is a fundamental characteristic of SHA's health coverage arrangements, where SHA assumes the financial responsibility for uncertain future health events affecting the registered beneficiaries.

2.6 Under IFRS 17, an insurance contract exists when SHA agrees to compensate beneficiaries if a specified covered health event adversely impacts them.

2.7 The Standard states that;

The uncertainty (or risk) is the essence of an insurance contract. Accordingly, at least one of the following is uncertain at the inception of an insurance contract:

- (a) the probability of an insured event occurring.
- (b) when the insured event will occur; or
- (c) how much the entity will need to pay if the insured event occurs.

Significant Insurance Risk

- 2.8 The concept of an insurance contract includes the notion of significant insurance risk, which occurs when an insured event requires the insurer to pay significant additional amounts in any scenario which has a commercial substance.
- 2.9 The Standard defines insurance risk as;

The definition of an insurance contract requires that one party accepts significant insurance risk from another party. IFRS 17 defines insurance risk as 'risk, other than financial risk, transferred from the holder of a contract to the issuer'. A contract that exposes the issuer to financial risk without significant insurance risk is not an insurance contract.

- 2.10 SHA should evaluate the contractual terms under each fund to determine if they meet the criteria for being classified as insurance contracts under IFRS 17.
- 2.11 For SHA, the significant risk lies in the uncertainty surrounding their obligation to pay out medical claims, including the unpredictability of the frequency and severity of those claims, and their overall financial impact.

Contracts Outside the Scope of IFRS 17

- 2.12 The scope of IFRS 17 does not encompass all contracts containing a risk element. Some contracts are considered to be outside the scope of IFRS 17.
- 2.13 Paragraph 7 describes certain contracts out of scope of IFRS 17 and their applicable accounting standards to account for them, as below;

- i. Warranties provided by a manufacturer, dealer, or retailer in connection with the sale of its goods or services to a customer (see **IFRS 15 Revenue from Contracts with Customers**).
- ii. Employers' assets and liabilities from employee benefit plans (see **IAS 19 Employee Benefits** and **IFRS 2 Share-based Payment**) and retirement benefit obligations reported by defined benefit retirement plans (see **IAS 26 Accounting and Reporting by Retirement Benefit Plans**).
- iii. Contractual rights or contractual obligations contingent on the future use of, or the right to use, a non-financial item (for example, some licence fees, royalties, variable and other contingent lease payments, and similar items: see **IFRS 15, IAS 38 Intangible Assets and IFRS 16 Leases**).
- iv. Residual value guarantees provided by a manufacturer, dealer or retailer and a lessee's residual value guarantees when they are embedded in a lease (see **IFRS 15 and IFRS 16**).
- v. Financial guarantee contracts, unless the issuer has previously asserted explicitly that it regards such contracts as insurance contracts and has used accounting applicable to insurance contracts. The issuer shall choose to apply either **IFRS 17** or **IAS 32 Financial Instruments: Presentation, IFRS 7 Financial Instruments: Disclosures and IFRS 9 Financial Instruments** to such financial guarantee contracts. The issuer may make that choice contract by contract, but the choice for each contract is irrevocable.
- vi. Contingent consideration payable or receivable in a business combination (see **IFRS 3 Business Combinations**).
- vii. Insurance contracts in which the entity is the policyholder, unless those contracts are reinsurance contracts held (see paragraph 3(b)).

- 2.14 By understanding these exclusions and focusing on the key elements of an insurance contract under IFRS 17 (*risk transfer, compensation for an uncertain event*), SHA can accurately assess which components of the underwritten insurance contracts and services offered by SHA are in scope of IFRS 17.
- 2.15 Some contracts may meet the definition of insurance contracts but are mainly meant to provide services for a fixed fee. An entity can apply IFRS 15 instead of IFRS 17 to these contracts if the conditions below exist:
- The price set is not based on the risk of each individual customer.
 - The contract provides compensation in the form of services, not cash payments.
 - The insurance risk arises primarily from the customer's use of services, and not the uncertainty about service costs.
- 2.16 An analysis of the SHA Funds is as shown in the table below;

Fund/Contract	Nature of Arrangement	Standard Applicable
Social Health Insurance Fund (SHIF)	Mandatory contributions pooled to cover healthcare costs for members. SHA bears insurance risk related to uncertain health events and service usage.	IFRS 17 – Insurance Contracts
Emergency, Chronic, Critical Illness Fund (ECCF)	Provides financial cover for specific, high-cost health events after SHIF limits are exhausted. There is a risk of uncertain events transferred to SHA. Funded through government allocations	IFRS 17 – Insurance Contracts
Primary Healthcare Fund (PHCF)	Provides primary healthcare services at no direct cost to beneficiaries. It is funded primarily through government allocations, not individual contributions.	IFRS 17 – Insurance Contracts
Public Officers Medical Scheme Fund (POMSF)	Provides medical cover to national and county government employees, constitutional office holders, and other eligible public officers.	IFRS 17 – Insurance Contracts

3 SEPARATION OF COMPONENTS FROM INSURANCE CONTRACTS

3.1 If an insurance contract held is made up of several parts that are managed separately and need to be measured independently to accurately reflect their rights and obligations, these parts should be treated as separate.

3.2 As per Paragraph 10 and 11;

10. An insurance contract may contain one or more components that would be within the scope of another Standard if they were separate contracts. For example, an insurance contract may include an investment component or a service component (or both). An entity shall apply paragraphs 11–13 to identify and account for the components of the contract.

11. An entity shall

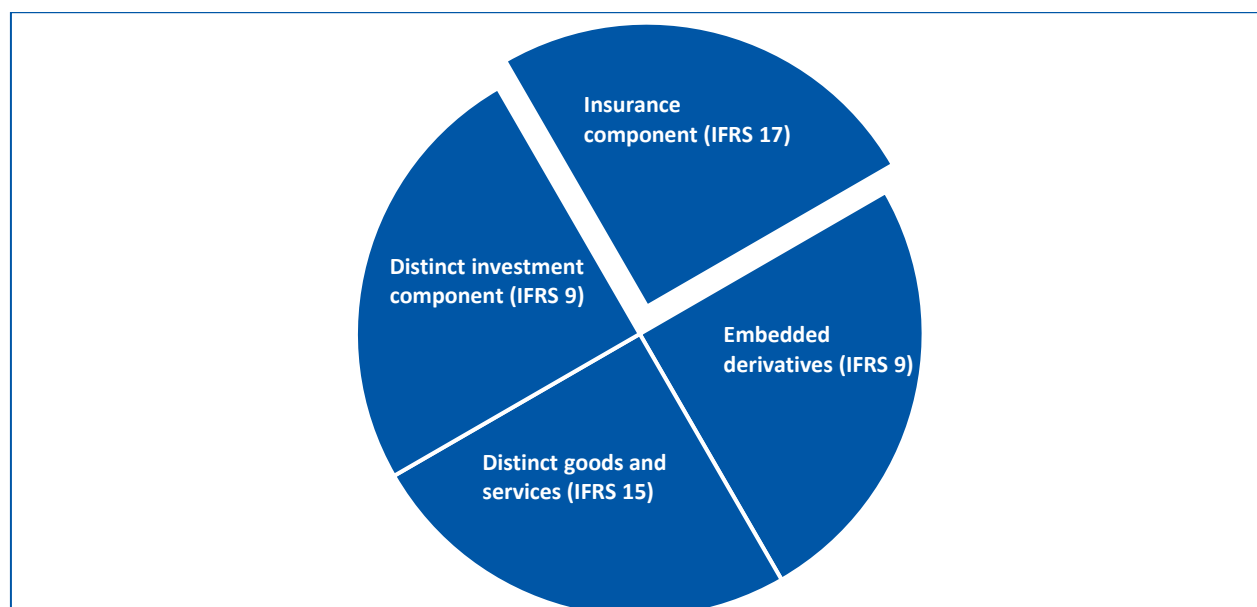
- a. Apply IFRS 9 to determine whether there is an embedded derivative to be separated and, if there is, how to account for that derivative.
- b. Separate from a host insurance contract an investment component if, and only if, that investment component is distinct

3.3 An insurance contract may include multiple components that, if considered separately, would fall under the scope of different accounting standards. Specifically, this applies to:

- a) Embedded Derivatives
- b) Distinct Investment Components
- c) Distinct Product / non- insurance service

3.4 IFRS 17 requires an entity to separate and account for non-insurance components embedded within the insurance contracts underwritten.

3.5 The chart below illustrates the separation of non-insurance components, if any, and how they are accounted for:



Embedded Derivatives

- 3.6 These are features within an insurance contract that behave like financial derivatives but are included as part of the contract *for example, interest rates.*
- 3.7 When these derivatives are not closely related to the insurance component, they need to be identified and accounted for separately.
- 3.8 An entity shall apply IFRS 9 to determine whether there is an embedded derivative to be separated and, if there is, how to account for that derivative.
- 3.9 SHA, however, does NOT issue any contracts which contain embedded derivatives, because contributions and payouts are based on health events and service costs, not market-linked variables.

Distinct Investment Components

- 3.10 These refer to portions of an insurance contract whereby a portion of the premiums is allocated to an investment or a savings account, which is separate from the insurance coverage itself.
- 3.11 Distinct investment components do not transfer significant insurance risk and can be separated from the main insurance contract.
- 3.12 If an investment component is distinct, it should be separated from the host insurance contract and accounted for by applying IFRS 9.
- 3.13 However, contracts issued by SHA do NOT have direct investment components. Members' contributions do not generate an individual investment balance, nor is there any guaranteed or repayable amount owed to members regardless of whether healthcare services are utilized.

Distinct Product / Non- insurance service

3.14 Distinct products refer to additional benefits provided under an insurance contract that are unrelated to insurance coverage. These could include services or products bundled with the insurance policy.

3.15 Paragraph 12 directs that;

After applying paragraph 11 to separate any cash flows related to embedded derivatives and distinct investment components, an entity shall separate from the host insurance contract any promise to transfer distinct goods or non-insurance services to a policyholder, applying paragraph 7 of IFRS 15. The entity shall account for such promises applying IFRS 15. In applying paragraph 7 of IFRS 15 to separate the promise, the entity shall apply paragraphs B33–B35 of IFRS 17 and, on initial recognition, shall:

- a. apply IFRS 15 to attribute the cash inflows between the insurance component and any promises to provide distinct goods or non-insurance services; and
- b. attribute the cash outflows between the insurance component and any promised goods or non-insurance services accounted for applying IFRS 15 so that:
 - i. cash outflows that relate directly to each component are attributed to that component;
 - ii. and any remaining cash outflows are attributed on a systematic and rational basis, reflecting the cash outflows the entity would expect to arise if that component were a separate contract.

3.16 A good or non-insurance service promised to a policyholder is distinct if the policyholder can benefit from it either on its own or together with other resources readily available to the policyholder.

3.17 After separating any cashflows related to embedded derivatives and distinct investment components, an entity should separate any promise to transfer distinct products to a policyholder from the host insurance contract.

3.18 SHA, however, does NOT administer contracts which include a promise to deliver a distinct product or service unrelated to insurance. For example, administrative services such as *registration* or *health promotions* are part of the public mandate, hence not separately sold as distinct products.

4 CONTENT COVERAGE

- 4.1 This section provides the main areas that will be addressed in IFRS 17.
- 4.2 Each subject outlines specific aspects of the Standard, detailing the specific requirements and methodologies for various elements of insurance contracts ensuring comprehensive understanding and implementation.

- 4.3 The main areas covered under IFRS 17 include:

- a) **Transition**- Provides guidance on how to transition from IFRS 4 to IFRS 17, including retrospective application and simplifications for easier implementation.
- b) **Level of Aggregation**- Gives the need for grouping insurance contracts into portfolios for the purpose of measurement and reporting in accordance with the similarity of their risk profile, the fact that they are managed together, profitability and year of issue (which should not be more than one year apart).
- c) **Initial Recognition and Contract Boundary**- This outlines the procedures adopted by insurers to comply with IFRS 17 requirements regarding the initial recognition of their insurance contracts and the contract boundary.

It provides detailed guidance on when the insurer begins recognizing insurance contracts and which cash flows are included within the contract boundary of insurance contracts. The contract boundary defines the cashflows that belong to a specific group of insurance contracts

- d) **Initial and Subsequent Measurement**- Highlights the techniques which need to be used to estimate the value of the insurance contract provided at the contract's beginning and at subsequent reporting. This involves different measurement methods; General Measurement Model (GMM), Premium Allocation Approach (PAA) and Variable Fee Approach (VFA).
- e) **Insurance Acquisition Cash Flows**- This section is related to the subject of how treatment of cash flows associated with acquiring insurance contracts, for example commissions and other costs, should be capitalized and amortized.

However, since every Kenyan household is mandated by law to contribute to the Fund, there are NO insurance acquisition costs incurred by SHA.
- f) **Risk Adjustment**- Involves measurement of a risk adjustment associated with the amount and timing of cash flows from insurance contracts to prescribe the amount of compensation that insurance enterprises need to receive for assuming the risk. SHA will include Risk Adjustment to enhance sustainability of the Fund.
- g) **Discount Rate**- Provides rules for the choice of appropriate discount rates to the cash flows generated in the future, coherent with the properties of the insurance liabilities and taking into account the time-value of money.
- h) **Modification and Extinguishment of Insurance Contracts**- Modification happens when the terms of an existing insurance contract are changed, for example, benefits and premiums or adding or removing a policyholder's rights or obligations.

Extinguishment of insurance contracts refers to the complete removal of an insurance contract from the financial statements of an entity.

5 RELEVANT IFRS 17 TEXT

Refer to IFRS 17 available online at the address below

<https://www.ifrs.org/content/dam/ifrs/publications/pdf-standards/english/2022/issued/part-a/ifrs-17-insurance-contracts.pdf?bypass=on>

This policy has been adopted and signed by the Board of Management of the Social Health Authority on this _____ day of _____ 2026.

Chief Executive Officer: _____

Signature: _____

Appointed Actuary: Ezekiel Macharia

Signature: _____

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